



P 3 2 · 1 6 P A G E S , P R I N T A B L E + F I L L A B L E

The Babysitter & Daycare Handoff Pack

Sixteen pages of handoff documents for daycare, the babysitter, grandparents, the date night, the parent's-night-out. The "everything they need to know" page, the food allergies and preferences page, the routine and signal page, the "in case of" emergency page, the consent forms, and a polite-firm what-we-do-and-don't-do page.

Soothemade Notes

A S M A L L A P O T H E C A R Y O F P R I N T A B L E S · M A D E S L O W L Y

A note before you print

Every time you leave your kid with someone, you have a small choice. You can re-explain the whole system, or you can hand them a page. This pack is the page.

Fill it in once. Update it when something changes (a new word, a new fear, a new allergy, a new bedtime). The work is in the first hour. After that, you are mostly handing it over.

With care, Maya

This is a planning tool, not medical advice. For specific medical or care concerns, please follow your pediatric provider's guidance. We do not name medications in this pack. Call the parent first, always.

How to use this pack

- Print on plain paper. The pack is meant to be marked up.
- Fill it in once, in pen, in the kitchen, with the partner if you have one.
- Make 3 copies. One on the fridge. One in the diaper bag. One in the car.
- Update once per season (4 times a year). Tape it to the calendar.
- For one-off handoffs (a date night, a parents' weekend away), hand the appropriate pages, not all 16.

The one-page summary

Tape this on the inside of the kitchen cabinet door. It is the page someone glances at first.

THE KID

Name: _____

Age: _____ years _____ months

Date of birth: _____

ALLERGIES (real ones)

☐ None

☐ Yes: _____

Response plan: _____

PARENT 1

Name: _____

Cell: _____

Work: _____

PARENT 2 (if applicable)

Name: _____

Cell: _____

Work: _____

PEDIATRICIAN

Name: _____

Number: _____

After-hours line: _____

EMERGENCY CONTACT (NOT the parents)

Name: _____

Number: _____

Relationship: _____

PICK-UP PERSON FOR TODAY

Name: _____

Number: _____

WHEN TO CALL THE PARENT IMMEDIATELY (not 911):

- The kid has hit their head and now is not acting like themselves
- The kid has a fever the caregiver isn't sure about

- The kid has a rash that is new
 - The kid will not stop crying after 20+ minutes
 - The caregiver just feels something is off
-

The full caregiver brief

For daycare, nannies, or anyone who has the kid for more than 4 hours.

THE FULL BRIEF

DAILY ROUTINE

Morning

Wake-up window: ____ to ____
Breakfast typically: _____
First nap (if any): ____ to ____
Morning play: _____

Midday

Lunch: _____
Quiet time / nap window: ____ to ____
Afternoon play: _____

Late afternoon

Snack: _____
Wind-down: _____

THE CURRENT SLEEP WINDOWS

Morning nap typical length: ____ minutes
Afternoon nap typical length: ____ minutes
"Wake window" before next sleep: ____ hours

THE CURRENT FEED / EAT SCHEDULE (approximate)

Breakfast at: ____ am Usually eats: _____
Snack at: ____ am Usually eats: _____
Lunch at: ____ pm Usually eats: _____
Snack at: ____ pm Usually eats: _____
Dinner at: ____ pm Usually eats: _____

THE CURRENT OBSESSIONS

Favorite book: _____
Favorite song: _____
Favorite stuffed animal: _____
Favorite game / play: _____

THE CURRENT DISLIKES

THE WORDS / SIGNS THE KID USES

For "I'm hungry": _____

For "I'm tired": _____

For "I'm scared": _____

For "I need to potty": _____

For "I want my parent": _____

For their stuffed animal: _____

The food page

FOOD

ALLERGIES (in detail)

- ☐ None known
- ☐ Yes: _____
 - Severity: ☐ mild ☐ moderate ☐ severe (auto-injector required)
 - Symptoms: _____
 - Response: _____
 - If response needed, call parent AND ___ before anything else: _____

FOOD SENSITIVITIES (not allergies, but worth knowing)

- ☐ _____
- ☐ _____

CURRENT EATING STAGE

- ☐ Exclusively breast / bottle
- ☐ Just starting solids (purees, single foods)
- ☐ Eating most family foods, mashed or chopped
- ☐ Full table foods, occasional pickiness
- ☐ Full table foods, big appetite
- ☐ Self-feeding, refusing to be fed

WHAT WE OFFER (preferences, not rules)

- ☐ Whole-fat dairy
- ☐ Strained yogurt
- ☐ Whole-grain breads
- ☐ Plain fruit + veggies, prepared simply
- ☐ Limited added salt for children under 2
- ☐ No added sugar before age 2 (after, modestly)
- ☐ Other: _____

WHAT WE DON'T

- ☐ Sugary drinks (juice, sweetened milks)
- ☐ Honey before age 1

- ☐ Whole nuts, popcorn, hard candies (choking)
- ☐ Hot dogs in round slices
- ☐ Whole grapes / cherry tomatoes (cut in quarters)
- ☐ Foods directly from a stranger's mouth or fork
- ☐ Other: _____

IF THEY REFUSE A MEAL

- ☐ Offer the same thing again in 30 min
 - ☐ Offer something simpler (a cracker, fruit)
 - ☐ Don't force
 - ☐ Don't replace with sweets to compensate
 - ☐ Tell us at pickup so we can plan the evening
-

The “what we do / what we don’t” page

A page about household choices. Not rules, but expectations.

WHAT WE DO

- ☐ Sunscreen before outdoor play (SPF ____ +)
- ☐ Limit screen time to ____ minutes / day
- ☐ Outside time daily, weather permitting
- ☐ Read at least one book per day
- ☐ Wash hands before eating
- ☐ Brush teeth twice a day (we have the toothbrush and toothpaste in the bag)
- ☐ Use the kid's specific words for body parts
- ☐ Other: _____
- ☐ Other: _____

WHAT WE DON'T

- ☐ Phone photos of the kid posted publicly (you may take photos, please don't post)
- ☐ Kissing on the face
- ☐ Tasting food directly off an adult's spoon
- ☐ Tickling beyond what the kid is asking for
- ☐ Forced apologies between kids
- ☐ Withholding food as a consequence
- ☐ Spanking, hand-slapping, or any physical discipline
- ☐ Calling the kid "naughty", "bad", "stupid", "smart" (we don't label, we describe behavior)
- ☐ Promises of things parents haven't approved (e.g. "if you're good, we'll get ice cream")
- ☐ Other: _____
- ☐ Other: _____

DISCIPLINE APPROACH WE USE

- ☐ Redirection (for under 3)
- ☐ Natural consequences (e.g. "the toy goes in time-out, not the kid")
- ☐ A 1-minute calm break (per year of age, up to 5 minutes max)
- ☐ Naming the feeling first, addressing the behavior second

☐ Other: _____

The routine and signal page

The body cues. The words. The transitions.

HOW TO READ THE KID

WHEN THEY ARE TIRED, THEY:

- ☐ Get quiet
- ☐ Pull at their ear
- ☐ Suck their thumb
- ☐ Lay their head down
- ☐ Get more clingy
- ☐ Cry harder than the situation calls for
- ☐ Other: _____

WHEN THEY ARE OVERSTIMULATED, THEY:

- ☐ Cover their ears
- ☐ Go very still or very wild
- ☐ Start hitting / pushing
- ☐ Refuse to participate
- ☐ Bury their face in a parent / caregiver
- ☐ Other: _____

WHEN THEY ARE HUNGRY, THEY:

- ☐ Get whiney
- ☐ Get short-tempered
- ☐ Lose interest in play
- ☐ Ask for milk / bottle
- ☐ Lie on the floor and refuse to move
- ☐ Other: _____

WHEN THEY ARE SCARED, THEY:

- ☐ Hide
- ☐ Go silent
- ☐ Ask for parent loudly
- ☐ Pretend they aren't scared
- ☐ Cling to whoever is nearest
- ☐ Other: _____

THE TRANSITION RITUALS WE USE

Before nap: _____

Before bed: _____

Before leaving the house: _____

Before bath: _____

After tears: _____

The “in case of” page

For the moments that might happen.

IN CASE OF

A SMALL CUT OR SCRAPE

1. Calm the kid first.
2. Rinse with water. Pat dry. Apply a bandage if the kid likes bandages, skip if they don't.
3. Tell the parent at pickup. No need to call.

A FALL

1. Comfort, then assess. Most falls are nothing.
2. If the kid is acting like themselves within 5 minutes, no call needed. Tell parent at pickup.
3. If the kid hit their head AND is acting sleepy, vomiting, confused, or otherwise off, call the parent immediately.

A FEVER (per touch, not measured)

1. Tell the parent. Always.
2. We do NOT give any medication without the parent saying so. Please call us.
3. Keep the kid in a quiet, cool space.

A RASH

1. Tell the parent. Always.
2. We do not apply any cream or lotion without the parent saying so.
3. Watch for: spreading, blistering, fever, or the kid scratching open the skin.

VOMITING

1. Comfort the kid.
2. Offer small sips of water once the vomiting stops for 15 minutes.
3. Call the parent.

THE KID WILL NOT STOP CRYING (more than 20 minutes)

1. Try the wind-down ritual. Try a hug. Try outside.
2. Try the favorite book.
3. If 20 minutes have passed and nothing is helping, call the parent.

THE KID HAS HAD AN ACCIDENT (potty)

1. Stay calm. Even pause-and-smile.
2. "It's okay. Bodies are learning. Let's get clean."
3. Use the spare clothes in the bag.
4. Put the wet clothes in the wet bag.
5. No need to call the parent unless asked.

THE KID IS HAVING A TANTRUM

1. Stay nearby. Don't try to reason with them mid-storm.
2. Try one of the cards from the Tantrum Scripts (P16) if you have them.
3. Wait. Tantrums end. Most last 5-15 minutes.
4. Repair after, when they are back in their body.

ANY EMERGENCY

1. 911 first if it's life-threatening.
 2. Call the parent SECOND.
 3. The pediatrician's after-hours line is also on the summary page.
-

The medication page

A page about medication that does not name any.

MEDICATION

WHAT THE KID TAKES DAILY (parent-approved only)

Medication 1 (by purpose, not name): _____

Time: ____ Amount: ____ Notes: _____

Medication 2 (by purpose, not name): _____

Time: ____ Amount: ____ Notes: _____

WHAT THE KID CAN TAKE "AS NEEDED" (parent-approved only)

For _____: parent will tell you when, and
where the bottle is.

RULE: WE DO NOT GIVE ANY MEDICATION WITHOUT THE PARENT
SAYING SO FOR THIS SPECIFIC INSTANCE.

If you think the kid needs something (a fever reducer,
something for an allergic reaction, anything else),
please call the parent first. Always. Even if it
seems urgent. Even if the kid has had this same thing
before. Always.

For genuine emergencies (severe allergic reaction
where the parent isn't reachable in time), follow
the response plan above OR call 911.

The consent forms

Two pages. Optional. Useful for daycare, longer stays, and trips with grandparents.

Medical-emergency consent

MEDICAL EMERGENCY CONSENT

I, _____ (parent name), give
_____ (caregiver name) authorization
to seek emergency medical care for my child,
_____ (child name), in the event
that I cannot be reached.

This authorization covers:

- ☐ Calling 911
- ☐ Transporting the child to an emergency room
- ☐ Authorizing emergency medical treatment until
I can be reached
- ☐ Sharing the child's pediatrician's information
with treating providers

This authorization is valid from _____ to _____.

CHILD'S DATE OF BIRTH: _____

CHILD'S PEDIATRICIAN: _____

CHILD'S INSURANCE: _____

Member ID: _____

PARENT SIGNATURE: _____

DATE: _____

(For a longer or more legally binding authorization,
talk to a lawyer or notarize the document. This
simple form is what most caregivers actually use.)

Drop-off / pick-up authorization (daycare)

DROP-OFF & PICK-UP AUTHORIZATION

I, _____ (parent), authorize the
following adults to drop off and pick up my child,
_____ (child name), from the
care of _____ (daycare / school
name):

1. _____

Relationship: _____

Phone: _____

2. _____

Relationship: _____

Phone: _____

3. _____

Relationship: _____

Phone: _____

NO ONE ELSE is authorized without my written approval
(by email or text from my registered number).

If someone other than these adults arrives, please
do not release the child. Call me first.

PARENT SIGNATURE: _____

DATE: _____

The daycare-orientation page

For when you are touring a new daycare or interviewing a nanny.

QUESTIONS TO ASK ON THE TOUR

ABOUT THE FACILITY (for daycare)

- ☐ What is the staff-to-child ratio?
- ☐ How is the facility cleaned and how often?
- ☐ What are the sick-child policies?
- ☐ How are vaccinations handled (theirs and other kids')?
- ☐ What's the food situation? Allergies handled how?
- ☐ Can I see the napping area? Outdoor area? Cubbies?
- ☐ What's the daily schedule, roughly?
- ☐ How do you communicate with parents?
- ☐ What's the discipline approach for tantrums, biting, hitting?
- ☐ Who is the lead teacher? How long have they been there?
- ☐ What's the turnover rate (politely asked)?

ABOUT THE NANNY (for an in-home caregiver)

- ☐ Tell me about your experience with kids age ____.
- ☐ How do you handle a tantrum?
- ☐ What does a typical day look like for you with a kid this age?
- ☐ What's your view on screen time?
- ☐ Are you comfortable with our food preferences?
- ☐ How do you handle a sick day (yours and the kid's)?
- ☐ What's your transportation situation?
- ☐ References. Three, minimum. Call all of them.

RED FLAGS TO WATCH FOR

- × Staff who do not look at or engage with kids during the tour
- × Crying children no one is attending to
- × Doors propped open into unsecure areas
- × No clear sick-child policy
- × Vague answers to specific questions
- × Pressure to commit immediately

- × Reluctance to share references (nanny)
- × Reluctance to let you observe (daycare)
- × Any feeling, in your body, that something is off

The "feeling something is off" one is real data.
Trust it.

The “fun things they like” page

A page so the caregiver can start a real connection.

THE KID, AT THEIR BEST

THEIR FAVORITE BOOK RIGHT NOW

THE BOOK YOU CAN ALMOST ALWAYS GET THEM TO LISTEN TO

THEIR FAVORITE SONG

THE SONG THAT CALMS THEM

THE GAME THEY LOVE

THE STUFFED ANIMAL / LOVEY THAT MATTERS

A THING THEY KNOW HOW TO DO THAT YOU CAN PRAISE

A WORD OR PHRASE THEY USE THAT MAKES US SMILE

WHAT MAKES THEM LAUGH HARDEST

WHAT THEY ARE LEARNING RIGHT NOW

THE THING THEY ARE SECRETLY GOOD AT

A final note for the parent

A page at the back, just for you.

Handing over your kid is hard. Even for an hour. Even to someone you completely trust.

You filled in this pack so that the handover does not require you to be in two places at once.

The kid will be okay. The caregiver, given this pack, has more information than most parents have on a normal day.

Walk to the car. Don't look back. Or look back once, smile, then walk to the car.

The day will not be perfect. Tomorrow you will do it again. It gets easier.

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From [Soothemade Notes](#), a small apothecary of printables, planners, and cards for the unphotographed parts of new parenthood. Made slowly, in plain language.



Soothemade *Notes*

*Made for the version of you no one tells you about, the
one at four in the morning, the one in the parking lot, the
one who doesn't recognize her own week.*

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